



Santa Clara Markkula Center for Applied Ethics

To: Department of Health and Human Services (HHS)
From: Santa Clara University's Markkula Center for Applied Ethics (Guadalupe Hayes-Mota)
Subject: Response to RFI: Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care (RIN 0955-AA13)

Response to the Request for Information on Accelerating the Adoption and Use of Artificial Intelligence in Clinical Care

Submitted by Guadalupe Hayes-Mota on behalf of the Markkula Center for Applied Ethics, Santa Clara University

The Markkula Center for Applied Ethics at Santa Clara University appreciates the opportunity to inform federal strategy for accelerating the responsible adoption of artificial intelligence in clinical care. Our work sits at the intersection of healthcare delivery, biotechnology, and ethics.

AI capability is no longer the primary constraint. Integration is.

Across the country, technically sound systems stall before routine use because of procurement timelines, uncertainty about liability, fragmented validation practices, and payment models that do not reward safe implementation. The central challenge for federal policy is institutional rather than computational.

To move from isolated pilots to systemic improvement, HHS can use existing statutory and programmatic authorities to create clearer expectations for transparency, oversight, and economic viability. Below we respond directly to priority domains raised in the RFI.

Specific Responses to RFI Questions

1. Barriers to Private Sector Innovation and Clinical Adoption

The major obstacles are operational friction and trust gaps.

Procurement versus model evolution. Health system purchasing and integration cycles can extend well beyond a year. During that time, clinical practices, patient populations, and data environments change. Without mechanisms for reassessment and updating, systems risk implementing tools whose original validation assumptions no longer match reality.

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Liability uncertainty. Health systems and clinicians remain unclear about how responsibility should be allocated when harm arises from systematic model error versus clinician judgment.

Workflow failure. Tools that increase alerts or cognitive burden without fitting existing care pathways are often ignored, even when technically sound.

2. Regulatory and Programmatic Priorities

HHS already possesses meaningful policy levers through ONC Health IT Certification, CMS payment policy, and FDA device authority. The opportunity is alignment and operational execution across these mechanisms rather than creation of new ones.

Recent reporting underscores why implementation details matter. A Reuters investigation examined post-market device reports associated with Johnson & Johnson subsidiary Acclarent's TruDi Navigation System after artificial intelligence capabilities were introduced. Reuters found that, following the AI addition, FDA received a substantially larger number of **unconfirmed** malfunction and adverse event reports, including allegations that surgeons were misinformed about instrument location during sinus procedures. The article also emphasized a critical limitation: Medical Device Reports are not designed to establish causality and may be incomplete.

Regardless of ultimate causation, these reports demonstrate an operational reality. Frontline users, purchasers, and health systems often do not have easy access to clear, comparable, version-specific information describing what changed, what evidence supports the current version, the contexts in which performance is validated, and the conditions necessary for safe use.

ONC Health IT Certification

45 CFR § 170.315(b)(11)

Current certification criteria for Predictive Decision Support Interventions already require extensive transparency through "source attributes." These include intended use, intended users, patient populations, development data characteristics including demographic representativeness, input features, external validation processes, quantitative performance measures, ongoing monitoring expectations, and update or maintenance information.



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However, these disclosures are frequently difficult to compare across products and are not consistently structured for operational decision making.

Recommended federal action

ASTP/ONC should translate existing disclosure obligations into a standardized, computable format. A model-card style artifact aligned with the National Institute of Standards and Technology AI Risk Management Framework could be machine-readable and consistently presented across vendors.

At minimum, it should identify:

1. the exact software and model version in clinical use
2. a clear history of modifications affecting AI functionality
3. validation scope for each version
4. known limitations, contraindicated uses, and foreseeable failure modes
5. required user competencies, training, and workflow dependencies

The central gap is not absence of authority. It is standardization, usability, and the ability of real organizations to operationalize what is already required.

FDA statutory framework

FDCA § 520(o)(1)(E)(iii)

Software may fall outside device regulation only if a healthcare professional can independently review the basis of the recommendation so that the professional does not primarily rely on the software's output. FDA guidance elaborates four criteria, all of which must be met, with independent review being pivotal.

For many modern machine-learning systems, what constitutes sufficient independent review remains uncertain for developers, purchasers, and clinicians.

Recommended federal action

HHS should promote tighter coordination between FDA and ASTP/ONC so expectations for transparency, explainability, and documentation are interpreted consistently across certification and device policy. Greater clarity would reduce regulatory arbitrage and market confusion.

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In safety-critical environments, FDA should further operationalize independent review through explicit expectations regarding human factors, training, labeling, and post-market monitoring when AI functionality is added to products already in use.

The Reuters findings regarding increased reporting following the TruDi AI introduction do not determine fault. They do, however, illustrate why version control, user-visible change communication, and actionable documentation are essential components of safe deployment.

Manufacturers of AI-enabled systems should therefore be expected to maintain and provide customers with version-specific safety materials, including update-triggered risk analyses, user-facing limitations, and monitoring plans capable of identifying performance drift or workflow confusion early.

3. Legal and Implementation Issues for Non-Device AI

For functions such as ambient documentation or generative drafting, the unresolved question is allocation of responsibility.

Vendor agreements frequently designate the clinician as the final authority. Yet in high-throughput environments, time pressure can limit meaningful verification.

Recommended federal action:

HHS can convene stakeholders and publish model contractual principles or safe-harbor approaches that distinguish:

- systematic or repeatable model defects, and
- patient-specific clinical judgment.

This would create predictability without requiring new legislation.

4. Evaluation Infrastructure

Today, validation is fragmented. Hospitals repeatedly test similar tools with limited resources, producing inconsistent evidence.

Recommended federal action:

HHS, including NIH and AHRQ, should support regional or federated evaluation collaboratives



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through cooperative agreements. These entities could test performance across diverse populations while allowing data to remain local.

A shared backbone reduces duplication and strengthens trust.

5. Reimbursement and Incentives

Payment policy heavily shapes behavior.

If AI saves time, adoption is straightforward. If AI improves quality but requires additional clinician effort to verify safely, organizations may be penalized financially.

Recommended federal actions:

- CMS should explore billing mechanisms or add-on payments that recognize clinician time spent validating complex algorithmic outputs.
- MIPS and other quality programs could reward documented governance practices such as bias auditing, monitoring, and incident reporting.
- CMS could evaluate pathways that allow qualifying Software as a Medical Device products to demonstrate value through reductions in utilization or complications.

6. Evidence from Successes and Failures

Federal policy should follow real-world performance, not marketing claims.

Example of relative success: Autonomous diabetic retinopathy screening achieved uptake in part because it fit a defined workflow and aligned with reimbursement structures.

Example of caution: Independent evaluation of a widely used proprietary sepsis model found poor sensitivity and large numbers of false alarms. The lesson is straightforward. Performance in development environments does not guarantee effectiveness in live care settings.

Recommended federal action:

Strengthen expectations for post-deployment monitoring and transparent reporting of real-world outcomes.



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8. Interoperability and Data Standards

Without reliable metadata, learning health systems risk confusing AI-generated content with human-authored information.

Recommended federal action:

ASTP/ONC should continue work through USCDI+ and FHIR implementation guidance so that provenance information can identify which system generated a recommendation, under what version, and at what time.

10. Research Priorities

The most urgent gaps are in implementation science.

We need longitudinal evidence regarding automation bias, clinician trust calibration, and whether reliance on AI alters professional skill development over time.

The United States does not lack innovation capacity. It lacks predictable, repeatable pathways to deploy innovation responsibly.

By standardizing transparency under existing ONC authority, aligning interpretation of FDA's statutory CDS boundaries, modernizing incentives within CMS, and investing in shared evaluation infrastructure, HHS can transform trustworthy AI from aspiration into operational reality.

Respectfully submitted,

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